

# Executive Summary

## Autism Therapy Platform — Discovery Research

Prepared for: Center Director

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This document summarizes secondary desk research conducted across three workstreams: in-session data collection, patient enrollment & intake, and treatment plans, billing & follow-up. Findings are secondary evidence only. Primary fieldwork is required before any product decisions are made.

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This executive summary synthesizes secondary desk research across three workstreams for the Autism Therapy Platform (India) Discovery phase: **in-session data collection, patient enrollment and intake, and treatment plans, billing, and follow-up**. Research was conducted in April 2026 via web search and published sources. All findings are secondary evidence only. Primary fieldwork with center directors and therapists is required before any product decisions are made.

## 1. What We Know

### In-session data collection

- Real-time, in-session data recording is a clinical requirement in ABA — not optional. Retrospective recording degrades accuracy and is explicitly flagged as clinically inferior in peer-reviewed literature.
- Paper-based data collection has documented failure modes: miscounts, illegible notes, transcription errors, and delayed supervisor feedback. A peer-reviewed DTT study found measurable accuracy differences between paper and digital recording.
- Digital data collection is used in over half of ABA practices globally. All major platforms (Noteable, Raven Health, Ensora ABA) are built for one-handed, offline-first use — the established design baseline.
- No therapist-side clinical data collection tool has penetrated the Indian market. TherapEZ offers general practice management. Cognitivebotics tracks child performance within its own platform. Neither addresses therapist recording against individualized therapy program targets during live sessions — structurally different tasks.

### Patient enrollment and intake

- There is no standardized intake protocol for autism therapy in India. The process is fragmented, medically-mediated, and entirely paper-based.
- Families arrive at intake after a long diagnostic journey — often years of waiting. Intake is a high-stakes trust moment; disorganized intake is a documented driver of early dropout (described in research as "invisible exits").
- India's DPDPA 2023 creates real compliance risk: digital health data of minors requires verifiable parental consent. Most centers are currently non-compliant because data lives in informal tools and paper with no formal consent process.
- UDID documentation support is an underserved recurring need — centers regularly assist families with government disability ID applications, managed ad hoc today.
- No purpose-built intake tool for Indian autism therapy centers exists. US tools (SimplePractice, Jane App) assume HIPAA, insurance, and BCBA structures that do not apply in India.

### Treatment plans, billing, and follow-up

- Documentation burden is real and quantified globally: providers spend 2–3 hours per day on documentation in unautomated practices, reducible to 30–60 minutes with software. Whether this scale exists in Indian centers is unvalidated.

- Indian billing is informal and relationship-driven: fees collected via WhatsApp, handwritten receipts, and Excel. No insurance billing equivalent exists. Billing is a social act with emotionally stressed families — not a neutral administrative process.
- Appointment no-show rates vary dramatically by reminder method: 39% with no reminder, 24% with automated voicemail, 3% with live contact. Structured reminders are a high-ROI, evidence-based intervention with direct revenue impact.
- Dropout in Indian centers is driven by financial pressure, caregiver exhaustion, and communication breakdown. Most centers do not track dropout because no attendance system surfaces patterns early.
- WhatsApp is infrastructure, not a gap. Any follow-up or communication feature must work with WhatsApp behavior, not compete against it.

## 2. What This Means — Decisions Required

- **Which workstream is the primary entry point?** In-session data collection has the strongest clinical forcing function. Intake has the highest compliance urgency. Billing/follow-up has the most direct revenue impact. This decision must be made before primary research begins — it determines who we recruit and what we observe.
- **Is offline-first a hard requirement or a Phase 2 feature?** Secondary research confirms it is the global baseline. India-specific connectivity data in therapy settings is unvalidated. The answer determines architecture choices before engineering begins.
- **What is the minimum viable data collection interaction?** One-handed, ≤2-tap entry is established globally. We need to know which specific data types Indian centers actually record before designing the interaction model.
- **How do we position against WhatsApp for parent communication?** Any feature competing directly with WhatsApp will fail. The product must complement or integrate — this strategy must be set before design begins.
- **What is the right pricing structure for Indian centers (5–20 staff)?** Willingness-to-pay at Indian price points is entirely unvalidated. This is the most critical commercial assumption in the business case.
- **How do we handle DPDPA 2023 compliance?** It cannot be retrofitted. Consent architecture must be designed in from day one — especially for intake and health data of minors.

## 3. Open Risks and Assumptions

The following assumptions are driving current thinking. None have been validated through primary research. Each represents a decision point that fieldwork must answer.

| Assumption / Risk                                                                                                               | Level | Status                            |
|---------------------------------------------------------------------------------------------------------------------------------|-------|-----------------------------------|
| Special educators find paper/WhatsApp data collection disruptive enough to adopt software — even with behavior change required. | High  | Open — requires primary fieldwork |

| Assumption / Risk                                                                                                 | Level  | Status                                                      |
|-------------------------------------------------------------------------------------------------------------------|--------|-------------------------------------------------------------|
| Clinical supervisors spend significant uncompensated time on manual progress report and treatment plan writing.   | High   | Open — requires primary fieldwork                           |
| Center directors would pay for a unified tool at Indian price points if it materially reduced their admin burden. | High   | Open — requires director interviews and pricing research    |
| Offline-first is a hard requirement for the majority of Indian centers.                                           | High   | Open — no India connectivity data in therapy settings found |
| Centers using Cognitivebotics still rely on paper or WhatsApp for in-session clinical data collection.            | Medium | Open — requires center-level observation                    |
| Most Indian autism therapy centers are not DPDPA 2023 compliant for digital health data of minors.                | High   | Open — compliance risk; needs legal review                  |
| Intake dropout (families who make contact but do not enroll) is significant and unmeasured at most centers.       | Medium | Open — requires director interviews                         |
| Documentation in Indian centers takes 2+ hours/day (as documented globally in ABA practices).                     | Medium | Open — may be lower in less-structured Indian context       |
| WhatsApp cannot realistically be replaced for parent communication — the product must integrate or complement it. | High   | Open — requires parent and director fieldwork               |
| English-only UI is acceptable for therapist-facing features in metro markets at launch.                           | Medium | Open — requires onboarding observation                      |

## 4. Recommended Next Steps

These actions are required before any product or design decisions can be made. Secondary research has established the landscape — primary fieldwork must validate whether the problems exist at actionable scale in Indian centers.

| Action                                                                                                                                     | Owner                | Timeframe              |
|--------------------------------------------------------------------------------------------------------------------------------------------|----------------------|------------------------|
| Decide primary workstream entry point (in-session data, intake, or billing/follow-up) before primary research begins.                      | Center Director + PM | Before research begins |
| Recruit 5–8 center directors and senior therapists for 45-minute contextual inquiry sessions — observe actual workflows, not self-reports. | Researcher           | Week 1–2               |

| Action                                                                                                                                     | Owner           | Timeframe             |
|--------------------------------------------------------------------------------------------------------------------------------------------|-----------------|-----------------------|
| Observe at least 3 live therapy sessions to understand in-session data recording behavior directly.                                        | Researcher      | Week 2–3              |
| Conduct intake observation at 2–3 centers to map actual steps, timing, and documentation at each stage.                                    | Researcher      | Week 2–3              |
| Interview center directors on billing workflow: how fees are communicated, collected, tracked, and what happens when families fall behind. | Researcher      | Week 1–2              |
| Assess DPDPA 2023 compliance posture: engage legal counsel on consent architecture requirements for health data of minors.                 | PM + Legal      | Parallel to fieldwork |
| Test willingness-to-pay signal: present value proposition to 3–5 center directors and gauge pricing reaction directly.                     | PM              | Week 3–4              |
| Synthesize fieldwork findings into a validated problem statement before moving to Define stage.                                            | PM + Researcher | End of Week 4         |

**Stage gate reminder:** This document is the output of secondary Discovery research only. No solutions, features, or product decisions should be proposed until primary fieldwork is complete and a validated problem statement is approved. Moving to Define or Design without completing primary research is a stage skip.